CLINICAL ASSESSMENT TOOLS INCLUDE:

HOPE clinicians are trained on a variety of clinical assessment tools. The most commonly administered tools pulled into the CITP and Interim Assessment Reports, along with clinical interpretation, include the Sex Offender Treatment Readiness Scale (SOTRS - provided in screener package), the Criminal Sentiments Scale-Modified (provided in screener package), the LS/CMI (clinician administered), the STABLE-2007 (clinician administered), and the LOOK Assessment (clinician administered during the initial evaluation). Please note that the LOOK is only administered one time throughout the entirety of the clients' time at HOPE.

Other clinical assessment tools that can be administered, if clinically indicated, include the Bumby-Molest Scale, Bumby-Rape Scale, the Miller Social Intimacy Scale (MSIS), the UCLA Loneliness Scale, the Screening Scale for Pedophilic Interests (SSPI-2 - reserved for those who have committed at least one sexual offense against someone younger than 15-years old, based on self-report or criminal charge, and administered once); the Child Pornography Offender Risk Tool (CPORT - reserved for adult males with Child Sexual Exploitation Material (CSEM) offenses), the Sex Offender Treatment Intervention and Progress Scale (SOTIPS), the Desistence for Adolescents who Sexually Harm (DASH-13) tool, the Youth Needs and Progress Scale, the DSM-5 Cross-Cutting Measure; the University of Rhode Island Change Assessment (URICA), the Substance Abuse Subtle Screening Inventories (SASSI), Modified Mini Screen, Brief Situational Confidence Questionnaire, the Personality Inventory for the DSM-5 (PID-5), the Perceived Stress Scale, and Brief-COPE.

CLINICAL INTERPRETATION GUIDELINES (CLINICAL TOOLS)

These are specialized measures designed to help assess treatment progress and or to identify sexual interest and arousal. In most cases, completion of one or more such tests can be a requirement to advance through the various stages of the treatment program.

SOTRS:

The SOTRS helps to identify whether the client is ready for treatment now, moderately ready for treatment, somewhat ready for treatment, or not ready for treatment and in need of pre-treatment.

a. To score measure, simply add up all items and sum to obtain a total score

b. Interpretive guidelines are provided in TA

CRIMINAL SENTIMENTS SCALE-MODIFIED:

The Criminal Sentiments Scale-Modified (CSS-M) is a 41-item self-report measure designed to assess criminal attitudes organized into the following three content areas: (a) respect for the law and criminal justice system, (b) Tolerance of law violations; and (c) Identification with criminal others. The original CSS was rated on a 5-point scale until the test developers reworded several of the original items for clarity and changed the rating scale from five points to three points (1 = disagree, 2 = uncertain, 3 = agree) to form the CSS-M scale.

a. Please refer to the Criminal Sentiments Scale-Modified Scoring Key to determine the specific scores which correspond to Agree, Undecided, or Disagree.
b. Clinician adds up all items and reports total score. Subscale interpretation is not required
c. Total score Interpretation is as follows
    a. 22 and Below = Low level of antisocial attitudes
    b. 23-36 = Moderate level of antisocial attitudes
    c. 37 + = High level of antisocial attitudes

THE LOOK ASSESSMENT:

The LOOK Assessment is a computer-based idiographic assessment of sexual interests, including pedophilic sexual interest, which generates data for the preferences of different age and gender categories. LOOK combines a participant's opportunity to report his sexual preferences along with having three (3) covert viewing time validity measures. LOOK incorporates an anti-feigning tool that requires participants to look at pictures and make a decision concerning sexual attractiveness. As with any single assessment, the results of the LOOK should not be used in isolation to determine the presence or absence of any specific sexual interest. Click here https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/assessment-process/look-assessment for YouTube tutorials on how to administer and score the LOOK.

BUMBY - MOLEST SCALE:

The MOLEST scale (Bumby, 1996) is a 38-item self-report measure of cognitions, assumptions, and or beliefs supportive of sexual offending against children. Higher scores represent more justifications, minimizations, rationalizations, and excuses for sexual activity with children. To

score measure, simply add up all items for total score. Scores over 60 are considered clinically dysfunctional (Nunes, Pettersen, Hermann, Looman, & Spape, 2014); that is, are indicative of attitudes and beliefs that are supportive of sex with children.

BUMBY - RAPE SCALE:

RAPE scale (Bumby, 1996) is a 36-item self-report measure of cognitions, assumptions, and/or beliefs supportive of sexual offending against adults. Higher scores indicate more justifications, minimizations, rationalizations, and excuses for rape. Scores over 76 are considered clinically dysfunctional. To score measure, simply add up all items for total score. Scores over 76 are considered clinically dysfunctional (Nunes, Pettersen, Hermann, Looman, & Spape, 2014); that is, are indicative of attitudes and beliefs that are supportive of rape.

SARATSO AUTHORIZED RISK ASSESSMENT TOOLS:

HOPE is responsible for ensuring all clients convicted of committing a sexual offense are evaluated in accordance with SARATSO policy.

Static and Dynamic Risk Factors (DRFs) are factors statistically associated with increased recidivism risk and thus are targets for treatment.

STATIC-99R

Registered Sex Offenders (PC 290) – who are on formal supervision will have a completed Static-99R (provided by probation or parole) that will be reviewed and verified during the individual's onboarding. The Static-99R is an actuarial instrument designed to assist in the prediction of sexual and violent recidivism for clients. The Static-99R has shown moderate accuracy in ranking clients according to their relative risk for sexual recidivism. Furthermore, its accuracy in assessing relative risk has been consistent across a wide variety of samples, countries, and unique settings.

STABLE-2007/ACUTE-2007

The STABLE-2007 is an empirically informed, cross-validated, risk assessment instrument that relies on changeable factors to estimate risk of sexual recidivism. Hanson and Harris (2000, 2007) developed this risk assessment instrument based on a large prospective study from Canada and the states of Alaska and Iowa with a total sample size of 997 sexual offenders. The presence

of dynamic risk factors is not considered to aggravate an individual's risk; instead, these factors help identify protective factors or treatment targets that could potentially reduce or influence one's risk to engage in sexually harmful/illegal or problematic behavior. The STABLE-2007 is designed to track changes in different types of dynamic factors, which are potentially changeable, but persist in the long term (e.g. alcoholism, intimacy deficits, etc.).

The ACUTE-2007 is a specialized risk assessment designed to track changes in different types of acute dynamic factors, which can change in the short term, such as weeks or days, and signal the timing of new offenses (e.g. drunkenness, acute distress, etc.).

LEVEL OF SERVICE /CASE MANAGEMENT INVENTORY (LS/CMI)

The Level of Service Case Management Inventory (LS/CMI) is a risk and need assessment tool. The General Risk/Need score reflects the literature's "big eight" risk/need factors, which are Criminal History, Education/Employment, Family/Marital, Leisure/Recreation, Companions, Alcohol/Drug Problem, Pro-criminal Attitude/Orientation, and Antisocial Pattern.

REPORT DEADLINES

We are held to strict deadlines for all assessments and reports. See the timeline for the completion of reports below:

- **Comprehensive Individualized Treatment Plan (CITP)**-Due 30 calendar days from the initial referral (received date)
- **Sex Offender Specific Evaluation (SOSE)-**Due 10 calendar days after the second clinical interview hour
- **Psychosexual Evaluation-**Due 10 calendar days after the second clinical interview hour
- **Substance Abuse Assessment Report-**Due 10 calendar days after the second clinical interview hour
- **Mental Health Assessment Report-**Due 10 calendar days after the second clinical interview hour
- **Annual Assessment Report**-Update **once per fiscal year** (July 1 to June 30).
- **Interim Report**-Update **once per fiscal year** (July 1 to June 30) or **as needed** if additional measure(s) are clinically indicated.

DISCHARGE REPORTS

Discharge (DC) reports are to be completed for every client (CLT) who has entered treatment and subsequently is no longer participating in treatment at HOPE.

This includes but is not limited to clients who:

- Client completes their required parole/probation period
- Return to Prison
- At the request of supervising officer/agent
- Transfer to another contract/county
- Deceased

All DC summaries are to be completed in TA within five **(5)** calendar days of the Controlling Discharge Date (CDD). DC summaries must be approved by the supervisor on day seven **(7)** and received by our Stakeholders on day ten **(10).** If a client was never seen by HOPE's clinical team or if the client was referred for assessment only, the local OC or OM are to create an administrative discharge report.

The clinical Discharge Summary should include an accurate representation of the CLT's entire experience in treatment, not just the current clinician's experience. Clinicians are to perform a thorough chart review and consult with other clinicians to obtain an accurate picture of a particular client's journey in treatment. DC summaries require an updated risk profile as assessed by utilizing validated assessment tools. All known risk factors or high-risk behaviors must be reflected in the discharge summary, as well as the most recent polygraph results, reason for conclusion or termination of contracted services, a Diagnostic and Statistical Manual of Mental Disorders (DSM) diagnosis, progress in treatment goals or lack thereof; familial or prosocial supports at time of discharge, and recommendations for continued therapy and management of sexual deviancies or risky behaviors.

Removal from Group Collection:

If a CLT is unavailable for treatment for more than two (2) weeks, the Office Manager (OM) will remove the CLT from their assigned group collection, which will allow room for a new referral or a CLT requesting group transfer. Upon return to treatment, the OM will reassign the CLT to his/her former group collection, if available. If the CLT's former group is not available, then the

OM will reassign the CLT to another group collection, preferably with their former clinician. If the CLT's former clinician does not have an opening in any of his or her groups, then the OM will reassign the CLT to another group collection, cc'ing the former clinician and newly assigned clinician to ensure continuity of care.

Discharge Summary Example:

Mr. Doe is a 56-year-old Caucasian, heterosexually identified man who was referred by the California Department of Corrections and Rehabilitation (CDCR) for a 290-risk assessment and sex offender treatment. On November 28, 2012, Mr. Doe was convicted of Indecent Exposure, with prior convictions of Indecent Exposure and Solicit a Lewd Act on July 17, 1992 and July 18, 1995.

Diagnostic and Clinical Impression

At the time of Mr. Doe's intake evaluation on May 1, 2018, his assessor diagnosed him with cocaine dependence, uncomplicated, and other stimulant dependence, uncomplicated. Throughout treatment at HOPE, Mr. Doe never spoke of symptoms consistent with any other diagnosis; however, near the end of his treatment, he reported that he was going to see a psychiatrist for symptoms consistent with schizophrenia, bipolar type. While at HOPE, Mr. Doe was never outwardly symptomatic, and he was not on medications for this condition. Mr. Doe began treatment at HOPE on May 1, 2018, and he was assigned to a High-Risk Sex Offender (HRSO) group, in which he attended group sessions twice a week and individual sessions once per month. Mr. Doe missed seven appointments during the period in which he received treatment. He actively engaged in treatment sessions and he made progress towards his treatment goals. Specifically, Mr. Doe occasionally engaged in verbal impulsivity and vulgarity, and he often presented with heightened anxiety and excitability; however, despite these areas of concern, he was able to demonstrate progress by implementing behavioral control skills in session.

Furthermore, Mr. Doe demonstrated insight and knowledge of the materials presented in sessions. He was also able to identify the skills needed to initiate and maintain healthy relationships with prosocial peers and family members. He was also able to utilize his emotion regulation skills and maintain his sobriety. In regards to substance use, Mr. Doe identified past

use as a precipitating factor to his indecent exposures, and he acknowledged that if he were to relapse, he would increase his likelihood of repeating a sexual offense. Mr. Doe denied urges to return to substance use, and he maintained his sobriety throughout his required parole period.

When reviewing his discharge plans, Mr. Doe reported that he would continue to abide by three of the acronyms learned in sessions. First, to avoid impulsivity, Mr. Doe reported that he would STOP, THINK, ACT, and REVIEW (STAR) his thoughts and behaviors. To avoid risky situations, Mr. Doe said he would avoid SEEMINGLY UNIMPORTANT DECISIONS (SUDS), and last, to ensure he is engaging in healthy relationships, he reported that he would remember that there must be CONSENT, EQUALITY, RESPECT, TRUST, and SAFETY (CERTS) before engaging in sexual relationships. This clinician encouraged Mr. Doe to continue to practice these skills, as well as maintain his sobriety, as he completed his required parole period and was no longer eligible for the HOPE program. Mr. Doe's discharge date was 12/15/18. Monthly maintenance therapy was suggested.

Risk to Re-Offend

Scientific research has consistently shown that actuarial methods of risk assessment are more accurate than unstructured clinical judgements alone in predicting re-offending among individuals released into the community. Actuarial estimates are not perfectly accurate, but they do provide the best available estimate of risk for future sexual and non-sexual violence over the long-term.

Mr. Doe's overall general Risk/Need Total score derived from the LS/CMI was 27, which fell in the high level. Comparable to others, individuals with the same LS/CMI score as Mr. Doe have approximately 44% chance of re-offending within one year.

The STABLE-2007 can be combined with the Static-99 to provide a composite assessment of risk/needs, and to produce estimates of sexual recidivism, sexual recidivism with sexually motivated breaches, violent recidivism, and any criminal recidivism both with and without breaches. Mr. Doe's composite assessment placed him in level IVb for supervision and intervention using the standardized risk framework. Individuals at Level IVb are considered *Well Above Average Risk* using the standardized risk level framework. They often

have many criminogenic needs, most of which are chronic and severe. Access to prosocial resources and strengths are likely limited due to significant barriers.

The rate of sexual reoffending for individuals in Level IVb is about three to four times the average rate of reoffending for the overall population of individuals convicted of sexually motivated offenses. Most will transition to Level III after a sufficient dose of treatment, positive life changes, or should they remain offense-free in the community for 10 to 15 years.
The risk of reoffending can be described using risk ratios, which summarizes the risk of a particular Level compared to the average individual convicted of a sexual offense. On average, individuals in Level IVb are expected to have roughly three to four times the rate of recidivism compared to the average individual convicted of a sexually motivated offense.

The risk of Mr. Doe offending can also be described by examining the absolute rates of reoffending over a time period. Out of 100 individuals convicted of sexually motivated offenses with the same risk profile as Mr. Doe, Level IVb would be expected to recidivate sexually at a rate of 21.7% after 1 year, 28.8% after 3 years, and 33.7% would be expected to recidivate sexually (including sexually motivated breaches) over 5 years.

Recommendations

It is recommended that Mr. Doe continue monthly maintenance therapy to assist in the practice of healthy coping skills and maintaining his sobriety. He was also encouraged to gain a sponsor and attend a support group, such as Alcoholics Anonymous (AA) and Narcotics Anonymous (NA). Lastly, since Mr. Doe endorsed symptoms consistent with a mental health diagnosis shortly before discharge from HOPE, he was encouraged to follow up with a psychiatrist; an appointment was made and confirmed before discharge.

Thank you for this referral. If you have any questions, please do not hesitate to ask.

County Probation Clients:

Discharge summaries are to be completed for every client that leaves treatment. This includes clients that graduate from treatment and for those that are discharged for other reasons like non-compliance or a probation violation. The discharge summaries are to be completed in TA and the individual supervisor should be notified that the discharge summary is ready for review. This

must occur within **five (5) business days** of the client's last session. The discharge summaries are to be sent to the client's supervising officer **within ten (10) days** of the client's last session.

PROOF OF PRACTICE (POP)– APPOINTMENT SLIPS

Per policy, all clients receive an appointment slip for every appointment they are scheduled to attend. These are commonly known as Proof of Practice slips. Clients receive one appointment slip for their group schedule. Repeating group appointments only require one original appointment slip. The client only needs to receive a new appointment slip if their group schedule changes and/or if they request a new one.

A client is to receive a new appointment slip for every individual session, unless the client is being seen weekly, on the same/day time (i.e., recurrent sessions). In this case, the client only receives one appointment slip that states this is a repeating appointment.

It is imperative that every new appointment is accompanied with an appointment slip. The client receives the white copy and HOPE keeps the yellow copy. Proof of practice slips are used as reminders for upcoming appointments and proof of documentation that is often referenced for verification that the client was notified. Once both copies are signed by the client and clinician/operation member providing the slip, the POP is scanned and uploaded in the client's chart under documents in TA.

Policy

Individual appointments should be scheduled in a consistent fashion. If a participant is to be seen the 3rd Friday of every month, it should not change for the most part, unless rescheduled due to the appointment falling on a holiday. If a participant needs to move their appointment due to work or school change, the request MUST come from their supervising authority if the assigned clinician is unable to accommodate their schedule. In general, clients should attempt to schedule external appointments around their mandated treatment, which demonstrates their ability to problem solve and practice assertive communication skills with their employer.

Procedure Specifics

Appt. slips- Participants will get an appointment slip for individual sessions, their initial group session, group changes, polygraph examinations, and intake assessments. For individual

sessions, it is preferable that the clinician write a slip for the next session at the end of the current session and have the participant sign it so the clinician can upload it into TA chart.

Polygraph slips- As mentioned above, from time to time, participants will get notification slips for polygraphs. Polygraphs are mandatory. If a participant complains that they cannot come at that date and time scheduled, they must speak to their supervising authority and the supervising authority should request the appointment be changed or excuse them 72-business hours in advance. As mentioned above, the POP slip includes a portion for the participant to retain, and a portion for the file. The POP should be scanned or filed immediately to help with proof that the client was notified should they be absent.

Refusal to sign appointment slips- If a participant refuses to sign a slip, please write "refused to sign" on the slip and scan/file the slip into the client's TA file. Include a collateral note outlining your discussion with the client in his or her chart.

SIGN-IN SHEETS

FEDERAL CLIENTS

Per the US Federal Government, all federal clients must write the time of their treatment appointment in the space provided on the sign-in sheet. All sign-in sheets are to be signed and dated by the treating clinician and turned in to the Office Manager at the end of the month. If a client is absent from treatment, it should be noted if the absence is excused or a no-show on the sign-in sheet, on the right column. Please note that all no-shows must be excused by the supervising officer three business (3) days (Monday-Saturday) or 72 business hours prior to the session, in writing. Examples of excused absences include medical emergencies, prescheduled medical appointments, notification of incarceration or absconding supervision, and employment, but ONLY if HOPE's schedule were to interrupt a client's consistent work schedule, verified by their supervising officer or agent.

CLIENT ATTENDANCE AND ABSENCES

Consistency in treatment is paramount to managing risk and successful completion of a treatment program. Clients are required to attend treatment regularly and at the frequency per their

treatment plan. Unless the session is excused 72 business hours in advance for an approved reason by the supervising officer or agent, the missed session is to be marked as a "No-Show."

When a probation client reschedules their session to a new day/time, they are only billed for the one session they rescheduled. For example, if a client's upcoming session conflicts with a funeral for a family member, the client can request a switch 72-business hours in advance; however, the session can only be changed as a courtesy if the clinician has the bandwidth to reschedule their appointment within the same week. In this case, the client would only charged for one appointment when the protocol is followed correctly. If the clinician does not have the availability to reschedule the appointment, then the client would be marked as a no-show should they not attend the session.

Any missed session requires a Missed Session Alert (MSA). See Missed Session Alert section to learn about his process.

Excused vs. No-Show Absences

Absences can be considered excused if the supervising officer or agent notifies HOPE 72 hours in advance of the following qualified reasons:

- Status Updates (i.e., In custody, absconded, on medical leave, transferred, or discharged)
- Medical emergency, defined by being hospitalized or confirmation of contagious diseases (e.g. COVID-19 infection).
- Prescheduled medical appointments

Any other reasons for not attending treatment are considered No-Shows, even if "excused" by their supervising agent or officer. An MSA should include details outlining the client's reason for his or her absence.

Missed Session Alerts

HOPE Program will notify the supervising officer of any missed appointments by generating a written notification, known as a Missed Session (MSA) Alert, by the close of business on the same day of the missed absence. All missed appointments (**excused or no show**) are generated in TA and sent directly to the assigned supervising officer or court.

If a client has **two (2)** no-show absences, the treating clinician will place the client on a behavioral contract and request a meeting with the client and his/her supervising officer to address the client's inconsistency in attendance. This containment meeting is typically held at the client's individual session time. Upon completion of the contract, the treating clinician will email a copy to the supervising officer and a copy will be uploaded to the client's file in TA. If a client continues to be non-compliant with attending his or her mandated treatment after signing the contract, s/he can be terminated from the HOPE Program or violated by the supervising officer for noncompliance with mandated treatment. Once or if violated, the process starts over and the client will need to be placed on another contract should s/he miss another three sessions.

STATUS ALERTS

A Status Alert is a formal document that reflects an incident or disclosure that took place with a client. Status Alerts (SA) are sent to the supervising officer or agent before the close of business on the same day the event or disclosure took place, or when the clinician learns about the problematic or risky behavior. A courtesy phone notification can also be requested at HOPE's or the supervising officer's discretion. Phone calls made to or received by officers or agents are to be entered into a collateral note in the client's electronic file. Problematic or risky behavior includes, but is not limited to, the following:

- Suspected drug or alcohol use
- Aggressive behavior and or threats
- Suspected violation of the terms of probation or parole
- Any disclosure or concerns about high-risk behaviors
- Problematic behaviors in group or in the clinic
- Contact with law enforcement or police for whatever reason
- Contact with emergency services or Adult Protective Services (APS) or Child Protective Services (CPS)

A Status Alert can be emailed to the client's supervising officer any time the client is engaging in behavior that is deemed counterproductive to treatment. Sending a status alert should be discussed with your immediate supervisor, and the site's Clinical Director (CD), if necessary.

Upon advising one's supervisee to send a high-risk status alert for a client (in the case of a probation/parole violation, re-offense, an egregious program violation, etc., the supervisor must inform the site's Clinical Director (CD) of this decision.

PROGRESS NOTES

Any and all contact with a client is documented and **all progress notes are to be completed within 24 hours from the date of service.**

The note types HOPE uses are:

- Individual Progress Note
- Group Progress Note
- Collateral Note

INDIVIDUAL PROGRESS NOTE

The individual progress note is completed using the **GIRP** format. The GIRP format is best for active therapy sessions working off a detailed treatment plan. The I and R can be combined for simpler writing.

GOAL: The client's long-term goal and current focus of therapy. There may be more than one. The goals can be broad (e.g. reduce depression) or specific (e.g. increase daily communication with spouse) and may change throughout treatment.

INTERVENTION: Therapeutic interventions, actions, or methods used by the therapist to address goals and objectives during the session. What therapist did. This may be an interpretation offered, a clarification or information given, homework assigned, a challenge to narrow thinking about an issue, formal problem solving around the event, empathetic/supportive behavior on your part, functional analysis of a situation, a normalizing comment, or whatever is appropriate from the therapeutic conceptualization (i.e., theoretical approach) you are using. Also, included may be the therapist's understanding about the client's problem, the therapist's working hypotheses, the general content and process of the session.

RESPONSE: The response of the client to each of the therapist's interventions listed, as well as progress made towards treatment plan goals and objectives. This is also where the clinician may

add client's quotes, client's actions (e.g., yelled, cried, avoided), and client presentation (e.g., sad affect). Also included may be the client's current response to the treatment plan.

PLAN: Document what is going to happen next. What is the therapist and/or the client planning to do between sessions or during their next session? What is the therapist's plan for future treatment? What are the next steps in achieving treatment goals and objectives? What in the treatment plan needs revision? When is the next session date? The therapist might note whether they gave homework assignments, have topics to follow up on, or have actions to take before the next session (i.e., those can be entered here as reminders).

INDIVIDUAL NOTE TEMPLATE EXAMPLE:

Goal: To review and practice healthy coping skills and Cognitive Behavioral Therapy (CBT) techniques in response to endorsed stressors.

Intervention/**R**esponse: Mr. Doe arrived to his scheduled session on time, and he presented with an anxious mood and congruent affect, evidenced by self-report and psychomotor agitation. Discussion focused on Mr. Doe's recent reunion with his brother, whom he contacted via Facebook and text. This writer asked client whether he was allowed to use Facebook, as she believed it was against his conditions. Client reported that his supervising officer was aware. This clinician encouraged Mr. Doe to clarify with his officer as this writer would have to follow up; a STATUS ALERT was sent. Discussion continued by Mr. Doe reporting that his brother was dying of stage 4 liver cancer and cirrhosis of the liver. Mr. Doe also learned that his mother has no interest in reconnecting with him as a result of his criminal history. Mr. Doe processed his emotions and he endorsed feeling shame over his history of poor decision making. Empathetic and reflective feedback was given, and Mr. Doe was asked to challenge his negative self-talk by finding evidence against his cognitive distortions; client was receptive. Lastly, Mr. Doe and this writer discussed healthy coping skills and he practiced grounding techniques in session.

Plan: To review his conditions and practice at least three healthy coping skills before his next scheduled session, which is on Monday, 2/4/19. Mr. Doe was also scheduled to take his maintenance polygraph examination on Saturday, 2/2/19.

Risk factors reported: Violence: NONE; Suicidal: NONE; Engaging in risky or harmful sexual behaviors: NONE; Substance Use/Abuse: NONE.

GROUP PROGRESS NOTE

The group progress note is typed up in two paragraphs. The first paragraph should focus on the number of group attendees, followed by a brief overview of the topic (s) discussed during group session, while the second paragraph should focus on the individual group member being charted. Please note that no other client names or initials should be included in a client's chart.

Minimal Expectations in Each Group Entry:

As noted above, the group clinician will include the number of clients that attended session (e.g., 8 out of 9 clients) and provide an overview of the group. The group summary should include whether check-ins were performed, the topic (s) discussed (include phase of treatment and module), whether homework was assigned, and any significant announcements (e.g., group changes, polygraph reminders, or review of group rules, to name a few).

The second paragraph must include the client's presentation (i.e., mood and affect), their level of participation, their progress or lack of knowledge or insight in response to the curriculum presented, any homework assignments, and whether any risk factors were present. If any risk factors were present, the clinician will document the risk factors and any interventions applied, fill out the bolded template at the bottom of the progress note, and report whether a STATUS ALERT was sent.

GROUP NOTE TEMPLATE EXAMPLE:

7 out of 9 clients were present and on time for their scheduled group session. Today's session began with brief check-ins, review of last week's discussion on cognitive restructuring **(Core Block 1, Module 5)**, and an in-class assignment **(Homework 1.1.1)** on identifying the thought(s), feeling(s), and behavior(s) associated with a triggering event. Lastly, this writer announced that the time of group session would be changing from 1:00-2:30 pm to 1:30-3:00 pm, starting in two weeks; client's reported that they were okay with the change and proof of practices were provided.

Mr. Doe arrived to his group session on time, and he presented with a euthymic mood and congruent affect. Mr. Doe needed to be redirected on a couple occasions for interrupting his peers, but he actively engaged in discussion and was commended for demonstrating knowledge and insight when presenting his in-group assignment. Mr. Doe was encouraged to practice mindfulness and the behavioral control skills discussed during session. Mr. Doe appeared receptive to this writer's suggestion and reported that he would use his diary card to track his emotions over the weekend.

Risk factors reported: Violence: NONE; Suicidal: NONE; Engaging in risky or harmful sexual behaviors: NONE; Substance Use/Abuse: NONE.

COLLATERAL NOTES

Collateral Notes are another type of progress note utilized by the HOPE Program. Clinicians must create a collateral note in a variety of situations including any communication with an outside party (supervising officer, attorney, family member, etc.). The format for the collateral note can be a general narrative of what occurred. If the collateral contact is made via email, the body of the email, including the time stamp, is copied and pasted into the collateral note. For more detailed information on how and what to collateral, see steps 1-7 below. Please note, at a minimum, monthly collateral contact via phone or containment meeting with the supervising officer is required for every client on Probation or Parole.

<u>How and What to Collateral</u>

1. Communication with anyone outside of HOPE (e.g., parole or probation officers, clients, client's family or friends, attorneys) must be entered into a collateral note or via conversations in the client's chart in TA.
2. If the communication occurred in person or over the phone, paraphrase the conversation in your collateral note.
3. If the communication occurred via email or text message via RingCentral (never provide your personal cell phone number to clients), then copy and paste the whole text, to include the timestamp, date, and time of the correspondence in your collateral.
4. If emails include multiple client names, then omit all other client information and leave the name of the client you are documenting on in your collateral entry. Continue to do

this for each client discussed in your correspondence. Preferably, each client communication should be on a separate thread.

5. Do I collateral internal conversations with other HOPE staff? The answer is no, unless another staff was looped into your email to someone outside of HOPE.
6. What if a client calls to ask a simple question? The answer is that no matter how short or lengthy the call, it must be entered into a collateral note.
7. Lastly, when labeling collateral notes, be as specific as possible, although brief. For example, you would not label a note as a "phone call," as this does not provide enough detail to identify the contents in that entry. Instead, in this example, you could label it as "Phone call with AOR re: MSA." Other examples could be "Updated CDD," or "Client won't make it to group today."

CLIENT REPORTS VIOLATION OR BREAKING THE LAW

To ensure compliance with the Containment Model and our agreements and contracts with our stakeholders, any information disclosed that is a violation of the law and/or their conditions of parole/probation is reported to the supervising officer.

If a client reports in the waiting room or in session that they have violated their Terms and Conditions of Probation or Parole, or that they have broken the law, then we must report this information to the supervising officer or agent.

Procedure

Do not dismiss the client from the session, instead, continue with treatment. Ask the client if they reported their violation to their supervising officer. Remind them of the service agreement they signed upon entering treatment and of the Containment Model. This information will be reported to their supervising officer in a Status Alert. A Status Alert is warranted and is to be submitted on the same day the incident was disclosed to the agency. Be clear, concise, and direct in your documentation; state the facts. The progress note must also capture the disclosure and narrative of what events took place in the session and/or waiting room. Include what advice and information was provided to the client during the discussion.

COLLECTING PAYMENT

HOPE Program is committed to providing accessible and affordable services to all our clients. Following the service agreement outlined in the opening paperwork packet, all self-pay clients are obligated to make full payments for services at the time of service. Self-pay clients are defined as individuals who receive voluntary or mandatory services and who are required to pay at least a portion (i.e., up to 100%) of their treatment service fees. CDCR clients do not pay for their treatment and Federal Probation clients occasionally pay a co-pay for their treatment.

The process of collecting payment involves collaboration between our clinical and operations teams. Either before or at the time of the initial appointment, HOPE's operation team shall describe the fees for services to the client. If the client is a minor or lacks the capacity for consent, HOPE's operation team shall inform the parent or legal guardian of all fees. In the case of an emergency, the clinician shall inform the client of any fees as soon as is practical after rendering the service.

A variety of payment options are accepted, including Credit Cards (CC), debit cards, certified checks, money orders, and cash. Clients without CCs or bank accounts will have the option to pay via Check, Cashier's Check, and Cash. If paying via CC, the client is required to provide a signed Credit Card Authorization Form for auto payments, which will be processed when the client provides the Office Coordinator (OC) or Office Manager (OM) with the CC information. If the client does not possess a CC, they must pay dues before entering each session. Before sessions, the assigned or covering clinician will coordinate with the front desk (CC or OM) to confirm receipt of the client's payment. The messaging to clients when requesting that they pay their balance, in full, is to promote accountability and prevent the financial burden associated with accruing significant balances at HOPE.

If a client does not pay for services as agreed and if HOPE wishes to use a collection agency to collect the fees, HOPE Program will first give written notice to the client that a collection agency will be employed, but that the client will first be allowed a reasonable opportunity to make prompt payment. The written notice shall specify a date by which the collection agency will be employed if prompt payment is not received. HOPE shall limit information provided to the collection agency to that which is essential to recover the fees.

Finally, it may also be important to know that historically, despite clients being discouraged from accruing a balance, sometimes clients would accumulate one and HOPE would immediately place them on a behavioral contract and financial agreement if their balance reached $300.00; however, effective January 1, 2024, HOPE Program no longer permits the accumulation of financial balances for self-pay clients. If a former client returns to treatment and attends their first session without payment, then we can see them for the day, and emphasize that they will need to return the following week and pay their balance (current balance and next appointment) in full to be seen. If they return without payment, then they are dismissed, marked as a no-show, and discharged for non-payment. For more information on collecting client balances, visit the FAQ's here: https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/how-do-i-collect-outstanding-client-balances.

SCHOLARSHIPS

When a client enters the HOPE Program, unless otherwise noted, they are expected to pay the full fee for treatment. Scholarships are based on financial need and discussed when the client expresses financial hardship causing them to be unable to pay for treatment. When a client applies for a scholarship, they must first complete the budget homework and discuss the homework with their individual clinician. If after completing the homework the client still demonstrates need for a scholarship, the client can fill out the scholarship application and provide the necessary documentation (pay stubs, tax returns, etc.) and turn the application in to their individual therapist. The treating clinician then passes the application along their immediate supervisor.

For a client to be eligible for a scholarship they must demonstrate that they can maintain a zero balance for three (3) months prior to the scholarship being awarded. It generally takes three (3) weeks for a decision on eligibility for a scholarship.

Clinical Artifacts by Client Type:

The HOPE Program serves a variety of client types under several different contracts. While much of the clinical and administrative work is the same for all client types, there are some nuances and

differences between them. Please see below for a detailed account of clinical artifacts by client type.

Federal Clients – Pretrial

1. **Group and Individual Attendance: Same day** of service
2. **Group and Individual Notes:** Due within **24 hours** of services being rendered. See Documentation training for note templates
3. **Monthly Treatment Reports (MTRS):** Due to your supervisor by the **3rd** of each month. See Documentation training for minimum requirements
4. **Quarterly Treatment Plans (QTPS):** Due every **90 days**. See Documentation training for template
5. **Sex Offender Specific Evaluation (SOSE):** Only if ordered on the Prob 45 form. If ordered, the SOSE is due **10 calendar days** after the date of the last clinical interview appointment
6. **Containment notes (CTM notes):** Due **48 hours prior** to the scheduled CTM. The due date is assigned to the clinicians' task page in TA. All Federal Pretrial clients require a containment note **each month.** You are responsible for completing the CTM notes for all of your individual clients. If your client is in group only and you are the group facilitator, you are responsible for completing that note
7. **Discharge Summary: 5 calendar days** from date of discharge

Federal Clients – Post Conviction

All clinical artifacts for this client type are the same as the ones listed above **except for:**

1. **Annual Risk Assessments (Update Stable/LSCMI): Yearly** depending on the date of last update. Please refer to Stable/LSCMI training

CDCR Clients – State Parole

1. **Group and Individual Attendance: Same day** of service
2. **Group and Individual Progress Notes:** Due within **24 hours** of client's session. See Documentation training for templates

3. **Individualized Treatment Plan (ITP):** ITPs are a quick version of the CITP designed to be used when standing up a new site. It includes risk scores and self-report measures but no clinician narrative. ITPs are **due 10 days** from the scheduled interview appointment. See Documentation training for template
4. **Comprehensive Individualized Treatment Plan (CITP):** Due to supervisor **30 days** from the day the complete referral was received from parole, not from the scheduled interview date. The due date is assigned to the evaluator's task page by operations when the CITP is assigned. Please refer to CITP training for template and minimum requirements
5. **Containment notes (CTM notes):** Due **48 hours prior** to scheduled CTM. The due date is assigned to the assigned Clinician's task page. Group facilitators are responsible for completing the CTM notes for CDCR clients. If you have a client that is in individual only, then you, the individual clinician, are responsible for completing the CTM note.
6. **Treatment Completion/Discharge Summary: 5 calendar days** from date of discharge
7. **Interim Assessments:** Each CDCR client is required to complete **4 assessments per fiscal year (July 1 through June 30).** Two of those assessments must be the mandatory update of the Stable-2007 and LS/CMI. **Yearly** depending on the date of last update

Probation Clients (non-Federal), Social Services, Attorney Referrals

1. **Group and Individual Progress notes**: Due within **24 hours** of client's session
2. **Monthly Treatment Reports (MTRs):** Due to your supervisor no later than the **3rd** of each month, accounting for the previous month
3. **Annual Risk Assessments (Update Stable/LS/CMI): Yearly** depending on the date of last update
4. **Psychosexual Evaluation:** Due **10 days** after the last interview appointment.
5. **Containment notes (CTM notes):** Due **48 hours prior** to scheduled CTM. The due date is provided when the CTM email goes out to clinicians. Probation client CTM notes are completed by the individual clinician
6. **Discharge Summary: 5 calendar days** from date of discharge

Private Pay Clients

1. **Psychosexual Evaluation:** Due **10 days** after the last interview appointment.

2. **Group and Individual Progress notes:** Due within **24 hours** of services being rendered

PERFORMANCE IMPROVEMENT PROCESS

To ensure that we are providing quality care to our clients, All HOPE providers are responsible for documenting each client encounter completely, accurately, and on time. Clear and concise documentation is critical to forensic and clinical report writing as it vital to risk management and community safety. Accurate and timely documentation of all HOPE artifacts (e.g., client attendance, progress notes, alerts, and reports) will also help providers evaluate and plan the client's treatment, mitigate malpractice risks, maintain contractual compliance, and ensure payment for the services rendered. The practice of good risk management should build on the old adage of four Cs: **compassion, communication, competence, and charting**. In cases where a HOPE employee struggles with completing their documentation within HOPE's deadlines, staff will be placed on a Performance Improvement Plan (PIP).

PERFORMANCE IMPROVEMENT PLAN (PIP)

Purpose

A performance improvement plan (PIP), also known as a performance action plan, is a tool to give an employee with performance deficiencies the opportunity to succeed. It may be used to address failures to meet specific job goals or to ameliorate behavior-related concerns.

Initial Performance Concern

When a performance concern is identified or brought to a supervisor's attention the following procedures are intended to serve as a guide to supervisors while managing and developing employees with the task of executing their responsibilities. Some examples of performance concerns are: missing progress notes, status alerts, MTRs, interims, CITPs, or marking attendance.

Follow these steps when addressing performance concerns.

1. During the standing supervision appointment, the supervisor is to connect with the individual to address the concern within the same week the concern has been identified. If supervision has already passed, then a separate meeting is to be scheduled during the

same week. Addressing a concern in a timely manner gives more weight and significance to the concern. Typically, this meeting is all that is needed to address a concern. Failure to address concerns in a timely manner, often evolve into increasingly more problematic circumstances, in which it proves more difficult to correct.

2. Following the in-person meeting, an email summarizing the conversation and concern(s) is to be sent to the supervisee within 24 hours, explicitly stating the concern and purpose of the meeting and articulating a plan forward. This email needs to be saved in the employee file, therefore it is recommended that the email is forwarded to HR. Given this is the first written notification addressing a concern, it is not recommended to CC HR due to the direct implication of the appearance that this issue has already been escalated to HR. Ensure the supervisor document is updated accordingly and that the performance email is saved to the weekly supervisory meeting form.

    a. Click **here** for sample email template
    b. Click **here** for weekly supervisor meeting form template

    Note that if the staff member with performance concerns is licensed, then the CD, as opposed to the supervisor, is responsible for the aforementioned performance improvement steps. If for some other reason, the CD is not able to implement these steps, then the responsibility is delegated to the Executive Clinical Director.

Repeated Performance Concern

The next phase in this process is intended to address repeated concerns or ongoing performance issues. This step can be used if a repeated concern occurred within 12 months of it previously being addressed.

The steps are as follows:

1. The supervisor (or CD, if applicable) must create a list of the performance deficiencies, including dates, specific data or detailed explanations, and any previous guidance given to the employee.
2. Consult with the ECD and HR within 48 hours of the identified repeated concern.
3. Complete the **Performance Improvement Plan Document** (Click Here)

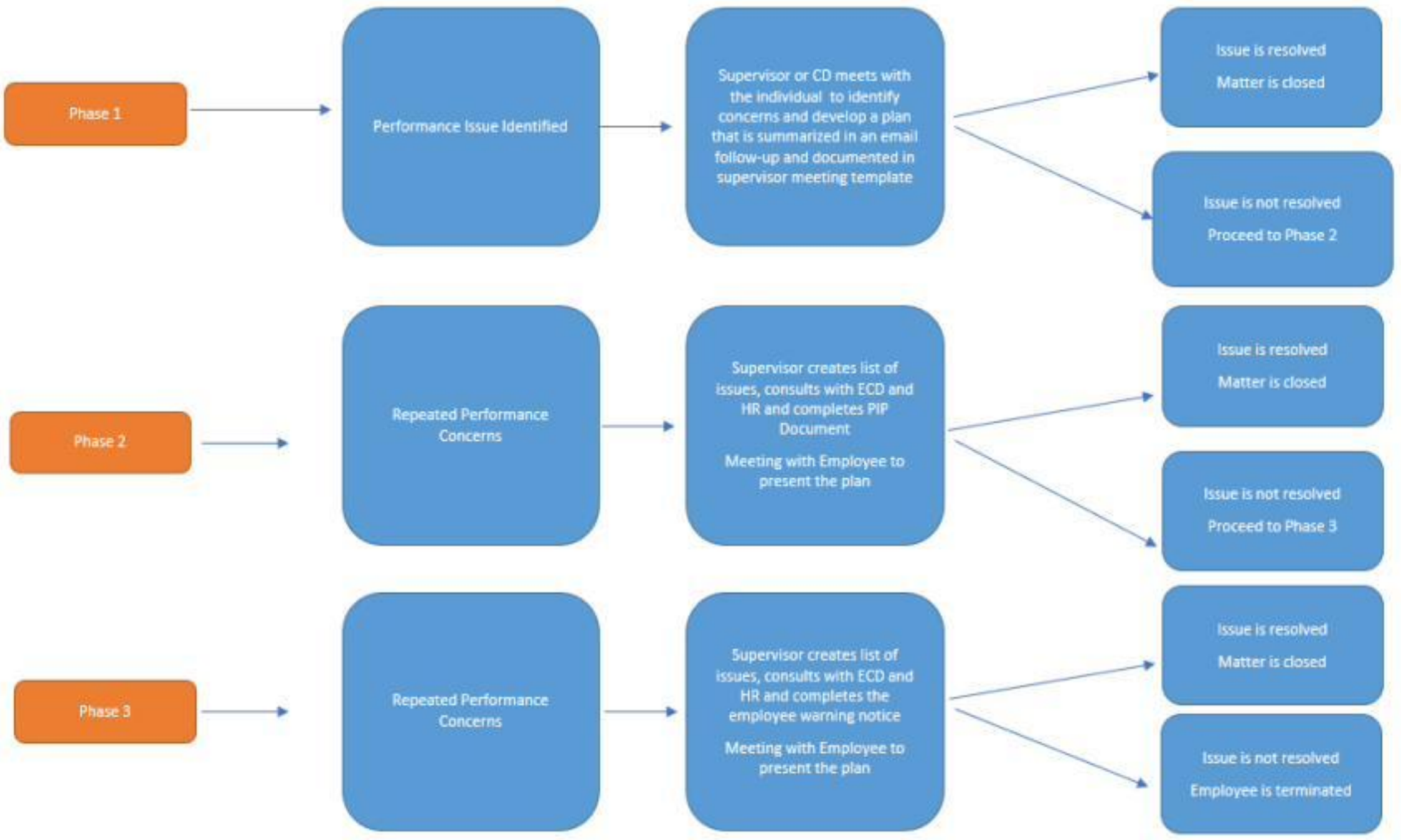
4. Schedule a meeting with the employee, the supervisor (or CD) and HR to present the plan

Final write up and/or termination

The third and final phase in this process is intended to addressing ongoing performance concerns while on the PIP.

1. The supervisor (or CD, if applicable) must create a list of the performance deficiencies, including dates, specific data or detailed explanations, and any previous guidance given to the employee.
2. Consult with the ECD and HR within 48 hours of the identified repeated concern.
3. Complete the **Employee Warning Notice** (Click Here)
4. Schedule a meeting with the employee, the supervisor (or CD), the ECD, and HR to present the write-up
5. If the identified issue is not resolved after this document has been presented, the employee is terminated. If the concerns have been adequately addressed, the matter is closed.

PIP Process Flow



USE OF POLYGRAPH EXAMINERS AND EXAMINATIONS

HOPE Program requires all clients enrolled in the Sex Offender Treatment Program to participate in one or more polygraph examinations while attending treatment at the HOPE Program. Generally, most clients take two polygraphs within the first calendar year (every 6 months), followed by one polygraph per year thereafter. All clients are required to complete a maintenance polygraph in order to be considered for graduation.

Any client can polygraph more frequently and as needed upon written approval from their supervising officer (probation), Unit Supervisor or Assistant Unit Supervisor (CDCR), or USPO (federal), accompanying an updated treatment plan. The written approval from stakeholders must be saved in the client's electronic chart.

POLYGRAPH EXAMINER QUALIFICATIONS REQUIREMENTS

HOPE Program works only with certified polygraphists. HOPE Program requires that the examiners performing the Clinical Polygraph are properly trained and adhere to the established ethics, standards, and practices of ATSA, and meet the certification standards adopted by the CASOMB, the American Polygraph Association, and the California Association of Polygraph Examiners.

Our examiners are required to complete a typed clinical polygraph report within 10 business days of conducting the examination. Clients undergo examinations at HOPE offices where they will meet with our subcontractor.

POLYGRAPH PROTOCOL

We require that all clients sign a specialized consent form stating that they agree to the Polygraph Examination and acknowledge that they understand the examination procedure. The Office Manager (OM) is responsible for coordinating the schedule with the polygraph examiner. Clinicians are responsible for tracking and ensuring that their clients are being polygraphed in a timely manner. The following types of polygraphs can be administered: An Instant Offense (IO) polygraph, a Sexual History polygraph, or a Maintenance/Monitor polygraph examination. However, due to an offenders elevated risk upon transition from prison or local custody into the community, a maintenance examination is recommended within the first six (3) months of treatment. Maintenance polygraphs will allow the polygraphist to ask questions that will focus on the offender's ability to comply with his or her conditions and restrictions of community supervision.

When a client participates in any polygraph examination, the examiner requires all the legal documentation, such as, but not limited to, police report(s), probation/parole report(s), which contains information as to what the client did or did not do during his or her instant or qualifying offense(s) and includes statements made by the client and the victim(s) to the police. Operations will follow up with probation or parole to ensure the required information is in their file. A description of the types of polygraph examinations HOPE Program utilizes can be found below.

TYPES OF POLYGRAPH EXAMINATIONS

SEXUAL HISTORY POLYGRAPH: Evaluates the truthfulness of the client's answers to their completed Sexual History Questionnaire. The test covers the client's sexual history prior to the date of his last conviction for a sexual offense. To assist in test question formulation, list three to four problem areas or specific areas where you suspect the client may not have been 100% truthful on the polygraph referral form (i.e. he has more victims than he disclosed, undisclosed female victims when he lists only male victims or vice versa, undisclosed adult victims when he lists only child victims or vice versa, involvement in a specific deviant behavior, use of force/weapons, masturbating to deviant sexual thoughts or fantasies or to infliction pain on victims, using pain for sexual arousal, etc.). The Sexual History Victims Test can also be used after the Disclosure over Sexual History Test has been completed to further address victims, types of sexual contact or use of force or weapons not reported.

MAINTENANCE POLYGRAPH: Used to determine the client's commitment to treatment by looking at his current behavioral markers in treatment and adherence to treatment program policies and terms and conditions of their parole/probation. Also, this test can be used to evaluate the client's truthfulness to his answers on a specialized Sexual Thoughts and Fantasies Questionnaire.

MONITORING POLYGRAPH: Used to address if a new SEX CRIME has been committed while on supervision or since a period of time on supervision. For example: During the last 6 months (or since starting supervision) have you had any sexual contact with a person you knew to be a minor?

SPECIFIC INCIDENT POLYGRAPH: Addresses the instant/qualifying offense(s) only. This test is most frequently requested when the client is in denial about their offense or any part of it.

SPECIAL REQUEST EXAMINATION: To ascertain clinically relevant information, i.e. behavioral issues, victimization issues related to patient's offense. State relevance of request; state referral questions.

PROCESS

HOPE Program schedules two polygraphs within the first calendar year (every 6 months), followed by one polygraph per fiscal year thereafter. After a client is identified to test, operations and polygraphers take the following steps:

<u>HOPE Operations Responsibilities:</u>

1. Ensures clients have a zero balance and have or will pay for the polygraph exam at least two weeks in advance (applies to self-pay clients only).
2. Ensures a Release of Information (ROI) for the polygraph examiner has been signed. The ROI should be updated yearly, or if the client is tested by a different examiner during the same year.
3. Obtain a Consent Form signed by the client to participate in the polygraph examination. This document should be updated yearly.
4. Create the polygraph sign-in and place it in the "Polygraph Sign-in" folder. For Federal clients, please ensure the service code is correct.
5. Schedule all clients around their existing services each month.
6. Send all polygraph referrals for external input by the 23rd of each month.
7. Referrals will be sent to the examiners by the 1st of each month.
8. Email the examiners about no-shows or excusals with directions on how to close out the referral.
9. Once the polygraph reports are complete, the Office Manager (OM) or Office Coordinator (OC) will email the report to the client's supervising officer, and cc the Clinical Director (CD) and treating clinician.

<u>Polygraph Examiner Responsibilities:</u>

1. Attendance, progress notes, and assessments must be completed by the close of every business day. Operations will reach out to examiners the following morning if any metrics are missing, with the expectation that they will be completed as soon as possible.
    a. Absences (i.e., no-shows) and excusals should be marked the same day examiners receive the email from HOPE.
2. Reports, videos, and federal charts are required to be uploaded within ten (10) days of the exam. Operations will reach out to examiners on day 11 if any are missing with the expectation they will be completed as soon as possible.

3. All documents must be labeled as follows:
    a. CDCR and County Probation reports: Last name, first initial (CDCR/PFN #) HOPE Polygraph Report [date of exam]
    b. CDCR and County Probation videos: Last name, first initial (CDCR/PFN #) HOPE Polygraph Video [date of exam]
    c. Federal Probation reports: First initial, last initial (PACTS #) HOPE Polygraph Report [date of exam]
    d. Federal Probation videos: First initial, last initial (PACTS #) HOPE Polygraph Video [date of exam]
    e. Federal Probation videos: First initial, last initial (PACTS #) HOPE Polygraph Charts [date of exam]
4. Examiners will close out the referrals by the 31st of each month.

- Clinical teams are not to schedule polygraph examinations
- Clinical teams do not create polygraph treatment modifications, nor do they request approval from parole to retest a client.
- Clients are only retested if they do not attend (i.e., no show) their exam and if the supervising officer specifically requests an additional test. In this case, operations would create the modification to gain written approval from the US/AUS.

COUNTY PROBATION

POLYGRAPH TESTING

Clients are required to polygraph within three (3) months of beginning treatment, and at the discretion of the treatment team and supervising officer thereafter. The client must provide a non-deceptive maintenance polygraph approximately three (3) months before leaving treatment/graduating. Best practices state that those who have graduated treatment should continue to polygraph every six (6) months for the duration of their time on supervision.

CDCR

POLYGRAPH TESTING

Clients are required to polygraph within three (3) months of beginning treatment. Clients will take two polygraphs within the first calendar year, followed by one polygraph per fiscal year

thereafter. The client must provide a non-deceptive maintenance polygraph approximately three (3) months before leaving treatment/graduating. Best practices state that those who have graduated treatment should continue to polygraph every year for the duration of their time on supervision.

FEDERAL

POLYGRAPH TESTING

Federal clients are required to polygraph every six (6) months for the duration for their time on probation. HOPE will require all new federal referrals to test within the first 3 months of entering treatment. USPOs must participate in the referral procedure and request what areas they want assessed during the polygraph.

CONREP

POLYGRAPH TESTING

CONREP clients are required to polygraph every six (6) months for the duration for their time on conditional release. HOPE will require all new referrals to test within the first 3 months of entering treatment. CONREP will participate in the referral procedure and request what areas they want assessed during the polygraph.

SPECIAL POPULATIONS:

HOPE Program provides services to a wide array of clientele. Only certain clients, such as private pay clients, can be denied services at HOPE and subsequently would be referred out to a qualified professional / program. If you have questions on eligibility, please contact your local Clinical Director (CD).

RECORD REQUESTS

California Department of Corrections and Rehabilitation (CDCR) Clients:

In the event that a client or their representative(s) requests a copy of client records, please know that the California Department of Corrections and Rehabilitation (CDCR) is the custodian of records. For further guidance on the process, see below:

Client Requests

1. Inform the client that CDCR is the holder and owner of client records, and direct them to CDCR to complete Form 7385, titled, "Authorization for Release of Protected Health Information."
2. Once the client completes the form at parole, the client can turn it in to parole's front desk administrator, and the CDCR representative will forward the form to the Sex Offender Unit (SOU). The SOU will review the request and respond accordingly.
3. Collateral all contacts, verbal or written, in the client's TA chart.

Representative Requests

1. Inform the representative that we cannot confirm or deny that the client they are requesting information for is in fact a client without a Release of Information (ROI).
2. Similar to above, inform them that CDCR is the holder and owner of client records, and direct them to call the local parole unit to request and complete Form 7385, titled, "Authorization for Release of Protected Health Information."
3. Once the representative completes the form, inform them that the CDCR representative should forward the form to the Sex Offender Unit (SOU). The SOU will review the request and respond accordingly.
4. If a representative sends you the completed form, forward the request to Janelle Harper (jharper@hopeprogram.biz) and cc your supervisor and Clinical Director (CD) for awareness.
5. Janelle Harper will forward it to the SOU, provide further instruction, and or loop in clinical if applicable.
6. If the representative's request involves information needed from the Sex Offender Treatment (SOT) provider (HOPE), the SOU will forward/work with their Division of Adult Parole Operations (DAPO) Litigation Unit (Northern or Southern Region depending on SOT vendor location), who will redact, log, and forward to the requestor.
7. If the request is for services the SOT vendors do not provide, such as the client's medical history (i.e. physical exam, mental exam, lab test, x-ray reports, etc.), then the SOU recommends that the SOT vendor (HOPE) respond to the requestor by advising them that

they Do NOT provide these types of services. This can be done simply by noting it on the form itself and sending it back to the requestor.

8. Collateral all contacts, verbal or written, in the client's TA chart.

Probation Client

In the event that a client (including self-pay probation clients) requests a copy of their records, please redirect the client to their Probation Officer (PO) so the PO can provide the client with the information they are requesting. The PO may request an updated Monthly Treatment Report (MTR) from the assigned clinician if not completed at the time of the client's request, but the PO would be the one to directly provide this to the client. The clinician is not to provide any documents to the client.

SUBPOENAS

In the event of a subpoena – contact your immediate supervisor to walk through the process and ensure the following steps are taken place within 24 hours. NOTE: All contacts, verbal or written, are to be documented in TA in the client's file.

Process:

1. Upload the subpoena to the client's file and enter a collateral note.
2. Forward a copy of the subpoena to Janelle Harper (jharper@hopeprogram.biz) with your supervisor and the Clinical Director (CD) cc'ed in that email.
3. Janelle Harper will forward the subpoena to the Sex Offender Unit (SOU) and cc the unit Agent of Record (AOR) and the Unit Supervisor (US). Again, ensure all communications are documented in TA/client's file.
4. Contact your supervisor via telephone. If your supervisor is unavailable, contact their immediate supervisor for guidance and to discuss the subpoena.
5. Contact the requesting agency/DA/Prosecutor – inform them that all client information belongs to the California Department of Corrections and Rehabilitation (CDCR) and HOPE is unable to release any information. CDCR is the custodian of records. In the event that the subpoena calls for testimony or presence in court, SOU recommends that HOPE contact the requesting agency, District Attorney (DA), and or prosecutor on behalf of HOPE, and ask if submission of documentation would suffice instead of an

appearance. If documents are accepted in lieu of an appearance, then the DA/prosecutor obtains the documents directly from CDCR/SOU. SOU's litigation department will connect with the requesting party and provide the necessary documents.

6. HOPE is prohibited from releasing any documentation to any requesting party at any time regarding CDCR clients.

ADDENDUMS

HOPE's Frequently Asked Questions

https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions

ATSA CODE OF ETHICS

https://www.atsa.com/atsa-code-ethics

CAMFT CODE OF ETHICS

https://www.camft.org/Membership/About-Us/Association-Documents/Code-of-Ethics

BBS

https://www.bbs.ca.gov/

BOP

https://www.psychology.ca.gov/

NASW CODE OF ETHICS

https://www.socialworkers.org/About/Ethics/Code-of-Ethics

APA Guidelines for Telepsychology

https://www.apa.org/practice/telehealth-telepsychology

A: PRIVATE CLIENT SERVICE AGREEMENT

HTTPS://DRIVE.GOOGLE.COM/FILE/D/0B0ORIUA7MY3KRJZDVXHIV1VVNG8/VIEW?USP=SHARING&RESOURCEKEY=0-_IKZMUOFWQZ38G0D-W5GTA

B: CONTAINMENT MEETINGS: PROGRESS REPORT & TREATMENT PLAN UPDATE PROGRESS REPORT

https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/containment-model-ctm/ctm-reports

C: POLYGRAPH CONSENT FORM

https://drive.google.com/file/d/0B0TrXm_ETls-WU5ZZXdjX2xLbzQ/view?usp=sharing&resourcekey=0-wFUozY5h7wR0nTaty00W8A

D: RELEASE OF INFORMATION

ROI-PRIVATE

https://drive.google.com/file/d/11UFDZ3SQ8KxPd1Wvk8XeJ62bbpQiJiRn/view

ROI-CDCR

https://drive.google.com/file/d/1nheVCNAMKF3ajDx3cg0o-hHxoUX0NqCx/view

ROI-Probation

https://drive.google.com/file/d/0B0TrXm_ETls-eHZFbUFDT3Vjb0U/view?usp=sharing&resourcekey=0-QupkxJ5FmmKtS8Qk1RryNw

E: Clinic Rules

https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/clinic-rules

F: Graduation Criteria

https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/hope-curriculum/hope-graduation-criteria

G: Treatment Plan Criteria

https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/assessment-process/treatment-goals

H: Training Library

https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/training

I: Articles

https://drive.google.com/drive/u/0/folders/1EpZJYJ6d1H-xEdFwwrtnVeFJG4tu9FY3

Telehealth Documents

https://drive.google.com/drive/folders/14cvC57pS1dqUwfguBUpUM0xf1Ifurt0x

HOPE Assessment Training Protocol Training

https://drive.google.com/file/d/1TWVVjaRkVCQbBaH1fDWw3HHjm4nNXy-k/view?usp=sharing

HOPE Best Practices Training

https://drive.google.com/file/d/1mZm9qP9yrqcOqZR_Owf4UI38FsSTpeCv/view?uCASdsp=sharing